

****HOSPITAL DISCHARGE SUMMARY****

****PATIENT DEMOGRAPHICS:****

Patient Name: Patient 41

Age: 68 years old | Sex: Male

MRN: 000-TEST-41

Admission Date: [ADMISSION DATE] | Discharge Date: [DISCHARGE DATE]

Attending Physician: [ATTENDING NAME]

****PRIMARY & SECONDARY DIAGNOSES:****

- Acute appendicitis with localized peritonitis — s/p laparoscopic appendectomy
- Chronic hypertension — controlled on current regimen
- Chronic Kidney Disease Stage 3a (baseline eGFR 48–52 mL/min/1.73m²)
- Hypokalemia (resolved)

****HOSPITAL COURSE****

Patient 41 presented to Metro Medical Center on [DATE] with RLQ pain, fever (38.9°C), and elevated WBC (13.2 K/uL). CT abdomen/pelvis with IV contrast confirmed acute appendicitis without perforation or abscess. Patient was consented for laparoscopic appendectomy.

****PROCEDURE DETAILS:****

Laparoscopic appendectomy performed without conversion to open. Three 5-mm trocar sites placed: one at umbilicus (camera), one at suprapubic midline (retraction), one at left midclavicular line (instrumentation). Appendix found to be erythematous with purulent exudate but no rupture. Appendiceal artery ligated with Harmonic scalpel. Appendix removed via

endoscopic specimen bag. All trocar sites irrigated with sterile saline. Fascial layer closed only at umbilical port; skin closure performed with 4-0 Monocryl subcuticular sutures. EBL minimal (~15 mL). Total operative time 38 minutes.

Post-operatively, patient tolerated NPO status initially with IV fluids (LR at 75 mL/hr). Diet advanced per protocol: clear liquids POD#1, advanced to full regular diet POD#2 without nausea or abdominal distension. Pain well-controlled with scheduled acetaminophen and PRN oxycodone. Trocar sites assessed TID — no erythema, seroma, or purulent drainage noted. Patient ambulating independently by POD#2. Potassium repleted (1 dose 20 mEq PO) for K+ 3.2 on morning labs. Discharged POD#3 in stable condition.

****DISCHARGE MEDICATIONS:****

Medication	Dose	Frequency	Duration
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Lisinopril	10 mg	q.d.	Continue indefinitely
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Hydrochlorothiazide	25 mg	q.d.	Continue indefinitely
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Acetaminophen	650 mg	Q6H PRN pain/fever	2 weeks or as needed
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Oxycodone	5 mg	Q4–6H PRN moderate–severe pain	1 week only
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Potassium Chloride	20 mEq	q.d.	4 weeks, then recheck serum K+
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****ACTIVITY & LIFTING RESTRICTIONS:****

- No heavy lifting, pushing, or pulling >5 lbs for 4 weeks post-op
- No strenuous exercise or high-impact activities (running, aerobics) for 6 weeks
- Light ambulation and ADLs as tolerated
- May return to sedentary desk work after 1 week; manual labor after 6 weeks

****DIETARY:****

- Regular diet as tolerated
- Adequate hydration (2–3 L/day) — important given CKD stage 3
- Avoid excess sodium (given HTN); <2g